NDIS Manual

2A: Implementing Behaviour Support Plans for One Life Foundation

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1- Applicable Practice Standards Policy & Procedure

1.1. Purpose

✓ The purpose of this policy & procedure is to sets out the relevant NDIS Practice Standards that apply to providers who implement behaviour support plans.

1.2. Scope

✓ This document applies to:

- All **One Life Foundation** staff and workers, whether permanent or casual, contractors, volunteers or business partners.
- All participants and their families

1.3. Definitions

Word/Term	Definition
Abuse	Abuse, refers to sexual assault, physical, emotional, financial and systemic abuse, domestic violence, constraints and restrictive practices and to neglect.
Aversion	An unpleasant stimulus (e.g. an unwanted cold bath, excessive chilli powder on food, liquid sprayed into a person's face etc.). Aversion is often used with the intention of manipulating behaviour. Such practices are prohibited.
Individualised Behaviour Support	The National Framework articulates principles and strategies for maximising individualised behaviour support for people with disability, with the overall objective of reducing the occurrence and impact of challenging behaviour and the use of restrictive practices. This may include the provision of positive behaviour support and development of an individual behaviour support plan. Positive Behaviour Support is the term used to describe the integration of the contemporary ideology of disability service provision with the clinical framework of applied behaviour analysis.

Word/Term	Definition
Behaviours of Concern	"Behaviour of such intensity, frequency or duration as to threaten the quality of life and/or the physical safety of the individual or others and is likely to lead to responses that are restrictive, aversive or result in exclusion." Any behaviour displayed by a person which is considered challenging or inappropriate by others, or which gives rise to reasonable concern, may be considered as challenging. However, the use of the term challenging should be understood in terms of the social context in which behaviour occurs, rather than a symptom of individual pathology.
Restrictive Practice	A practice that is used to restrict the rights or freedom of movement of a person for the primary purpose of protecting the person or others from harm.
Regulated Restrictive Practices monitored by NDIS Commission	There are five categories of regulated restrictive practices that are monitored by the NDIS Commission. These are the following: ➢ Chemical restraint ➢ Environmental restraint ➢ Mechanical restraint ➢ Physical restraint ➢ Seclusion
Seclusion	Seclusion, which is the sole confinement of a person with disability in a room or a physical space at any hour of the day or night where voluntary exit is prevented, or not facilitated, or it is implied that voluntary exit is not permitted.
Chemical Restraint	Chemical restraint, which is the use of medication or chemical substance for the primary purpose of influencing a person's behaviour. It does not include the use of medication prescribed by a medical practitioner for the treatment of, or to enable treatment of, a diagnosed mental disorder, a physical illness or a physical condition.
Mechanical Restraint	Mechanical restraint, which is the use of a device to prevent, restrict, or subdue a person's movement for the primary purpose of influencing a person's behaviour but does not include the use of devices for therapeutic or non-behavioural purposes.
Physical Restraint	Physical restraint, which is the use or action of physical force to prevent, restrict or subdue movement of a person's body, or part of their body, for the primary purpose of influencing their behaviour.

Word/Term	Definition
	Physical restraint does not include the use of a hands-on technique in a reflexive way to guide or redirect a person away from potential harm/injury, consistent with what could reasonably be considered the exercise of care towards a person.
Environmental Restraint	Environmental restraint, which restrict a person's free access to all parts of their environment, including items or activities.
Behaviour Support Plan (BSP)	A behaviour support plan is a document prepared in consultation with the person with disability, their family, carers, and other support people that addresses the needs of the person identified as having complex behaviours of concern. The behaviour support plan contains evidence-informed strategies and seeks to improve the person's quality of life. **Behaviour support plan** can be: (a) an interim behaviour support plan; or (b) a comprehensive behaviour support plan.
Behaviour Support Practitioners	A behaviour support plan can only be developed by practitioners who are considered suitable by the Commissioner to undertake functional behaviour assessments and develop behaviour support plans. They will be considered suitable pending assessment against the Positive Behaviour Capability Framework. Behaviour support practitioners (whether a sole provider or employed by a provider) must be registered with the NDIS to provide specialist behaviour support (Registration group 110).
Functional Behavioural Assessment	The process for determining and understanding the function or purpose behind a person's behaviour, and may involve the collection of data, observations, and information to develop an understanding of the relationship of events and circumstances that trigger and maintain the behaviour.
Specialist Behaviour Support Provider	NDIS provider whose registration includes the provision of specialist behaviour support services.
NDIS	National Disability Insurance Scheme

Word/Term	Definition
Person-centred	An approach which involves the person to gather information about that person's lifestyle, skills, relationships, preferences, aspirations, and other significant characteristics, in order to provide a holistic framework in which appropriate respectful and meaningful behaviour supports may be developed.
Prohibited Practice	Practices which interfere with basic human rights, are unlawful and unethical in nature, and are incompatible with the objects and principles of the Disability Inclusion Act 2014.
Consent	Permission given by a person with capacity to do so, or person(s) with legal authority to do so, on behalf of the person. For consent to be valid it must be voluntary, informed, specific and current. A person must be free to exercise genuine choice about whether or not to give or withhold consent, but it is only genuine if the person giving consent has the capacity and authority to do so.
Restrictive Practices Authorisation (RPA)	Endorsement for identified restrictive practices to be implemented with a certain individual, in a particular service setting, by associated staff and under clearly defined circumstances. Authorisation of a restrictive practice requires: • Behaviour support plan • Legal consent • Authorisation by an internal RPA Panel
Restrictive Practices Authorisation Panel	A panel comprising as a minimum, a senior manager of a NDIS provider and a specialist in behaviour support who is independent of the service provider. The RPA Panel authorises the use of a restrictive practice based on supporting information, such as a behaviour support plan.

1.4. Relevant Documents

- **National Disability Insurance Scheme (Quality Indicators) Guidelines 2020**
- **National Disability Insurance Scheme (Provider Registration and Practice Standards) Rules 2018**

- **The United Nations Convention on the Rights of Persons with Disabilities 2007,**
- **National Disability Strategy 2010–2020**
- **NDIS Quality and Safeguarding Framework**
- **National Disability Insurance Scheme Act 2013.**
- **National Disability Insurance Scheme (Restrictive Practices and Behaviour Support) Rules 2018**
- **National Disability Insurance Scheme (Provider Registration and Practice Standards) Rules 2018**
- **National Disability Insurance Scheme (Incident Management and Reportable Incidents) Rules 2018.**
- **National Framework for Reducing and Eliminating the use of Restrictive Practices in the Disability Service Sector 2014**
- **Disability Discrimination Act 1992**
- **The Privacy Act 1988 and the Australian Privacy Principles (March 2014)**
- **The Disability Discrimination Act 1992 (Cth)**
- **The Senior Practitioner Act 2018**

1.5. Policy & Procedure

1.5.1. Compliance Management

✓ **One Life Foundation** has established, implemented and maintained a process to identify legislative requirements and has access to all legal and other requirements that are applicable to **One Life Foundation**'s operations using **Form17.Legislative Compliance Register.**

✓ The register will be reviewed and updated at least once a year or after any major changes to the legislation. The evaluation will be carried out by the **Management Team** and the results will be reported to **Managing Director.**

✓ If there are any changes/ updates in the legislative requirements, relevant policies and procedures will be reviewed and revised accordingly.

✓ All Workers are responsible for managing compliance within their areas of influence by following the current policies and procedures. The changes and updates will be communicated to the workers and staff through meetings or receiving email updates.

✓ Legislative compliance is maintained and updated through:

- Reviewing NDIS Commission Website
- Ongoing consultation and communication with Industry Specialists/Legal representatives
- Legal updates provided by government publishing
- External third-party audit

2- Behaviour Support in the NDIS Policy & Procedure

2.1. Purpose

✓ The purpose of this policy & procedure is that each participant accesses behaviour support that is appropriate to their needs which incorporates evidence-informed practice and complies with relevant legislation and policy frameworks.

2.2. Scope

✓ This document applies to:

- All **One Life Foundation** staff and workers, whether permanent or casual, contractors, volunteers or business partners.
- All participants and their families

2.3. Definitions

✓ As per Applicable Practice Standards Policy & Procedure

2.4. Relevant Documents

- **NDIS Quality and Safeguarding Framework**
- **National Disability Insurance Scheme Act 2013.**
- **National Disability Insurance Scheme (Restrictive Practices and Behaviour Support) Rules 2018**
- **National Disability Insurance Scheme (Provider Registration and Practice Standards) Rules 2018**

- **National Disability Insurance Scheme (Incident Management and Reportable Incidents) Rules 2018.**
- **Form03.Improvement Report Form.**
- **Form09. Training Matrix form.**
- **Form10.Worker Performance Assessment.**

2.5. Policy & Procedure

Following the nationwide shift to the NDIS, providers now need to adhere to two tiers of regulations when implementing Behaviour Support Plans that include the use of restrictive practices:

- ✓ The Commonwealth NDIS Commission's regulatory framework, which outlines the situations in which restrictive practices can be used and how they should be reported, and
- ✓ The State's legislative framework, which outlines the approval and reporting requirements.

2.5.1. Positive Behaviours Support Policy

- ✓ **One Life Foundation** is committed to work towards the reduction and elimination of the use of restrictive practices.
- ✓ **One Life Foundation** is committed to make sure that restrictive practices should occur only in very limited and specific circumstances, as a last resort and utilise the least restrictive practice and for the shortest period of time possible under the circumstances. Restrictive practices should only be used where they are proportionate and justified in order to protect the rights or safety of the person or others.
- ✓ **One Life Foundation** is committed to that any participants who exhibit a cognitive or intellectual disability or any behaviour that causes, or has the potential to cause, harm is supported and monitored in a suitable environment that is aligned with the participant's unique needs.
- ✓ **One Life Foundation** is committed to support people's right, safety, and wellbeing in the

organisation. In fact, we believe that every behaviour has a purpose, and we should respect them and act properly to solve the issue. This includes people who put other people at any risk of harm and people diagnosed with mental illnesses.

✓ **One Life Foundation** will comply with n*ational Disability Insurance Scheme (Restrictive Practices and Behaviour Support)* Rules *2018 and* Disability Inclusion Act 2014 and The Senior Practitioner Act 2018.

✓ **One Life Foundation** is committed to ensure that our services meet agreed standards focussing on protecting and promoting the human rights of participants.

✓ **One Life Foundation** is committed to ensure that participants have protection against inhuman or degrading treatment and attention is provided to personal dignity, privacy and self-respect as well as individual needs.

✓ **One Life Foundation** is committed to ensure that our staff and workers work in a safe environment.

✓ **One Life Foundation** is committed to review the restrictive practices used (incident reporting if applicable), assessment of appropriateness and alternatives, and for aggregated reporting regularly.

✓ In cases that there is a challenging behaviour, positive behaviour support approach will be used by **One Life Foundation.**

2.5.2. One Life Foundation and staff Responsibilities

✓ **One Life Foundation** will:

- Ensure that all behaviour support practitioners and workers are assessed as suitable to deliver specialised positive behaviour support.
- Ensure that each behaviour support practitioner engages in ongoing professional development in the field of behaviour support, that is inclusive of evidenced-based support and best practice standards. This can be achieved by attending conferences and trainings.
- Ensure that each behaviour support practitioner has access to clinical supervision from

a specialist behaviour support clinical supervisor on a regular basis in relation to the provision of positive behaviour support practices.

- Ensure that workers are supported to develop and maintain the skills required to consistently implement the strategies outlined in behaviour support plans.
- Ensure that behaviour support practitioners are supported to train workers implementing behaviour support plans on how to implement the strategies contained within the plan.
- Ensure the availability, functioning and maintenance of all appropriate materials as well as equipment and that Behaviour Support Practitioner have appropriate training to use them.
- Ensuring that any change in practice recommendations is notified to all Behaviour Support Practitioner and workers.
- Review and implement any area of improvement using **Form03.Improvement Report Form**.
- Ensure all Behaviour Support Practitioners and workers participate in internal audit processes for nonconformities to be identified and action plans to be developed using internal audit report and improvement report forms.
- Share any learning from audits/incidents with Behaviour Support Practitioner and workers through meeting or via email updates

✓ **All Workers who implement behaviour support plans are responsible for:**

- Maintaining competency as per **Form09.Training Matrix** and **Form10.Worker Performance Assessment.**
- Attending relevant training provided by **One Life Foundation.**
- Bringing to the attention of appropriate senior staff any deficiencies in knowledge, ability, resources or equipment that may affect safe practice.
- Participating in audit programmes related to measuring the quality and safety of physical assessment and monitoring. This includes addressing any improvements required using improvement report form and celebration of good practice.
- Ensuring they are familiar with relevant policies and procedures in their area of practice using Worker Handbook and other relevant policies.
- Completing correct documentation.
- Reporting all incidents using incident report form.
- Using equipment as per manufacturers' guidelines.

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2.5.3. Training and Assessment of Staff

- ✓ Training plans will be developed and delivered using **Form09.Training Matrix** and through **Human Resource Management Policy & Procedure.**
- ✓ Workers who implement behaviour support plans must attend appropriate training and undergo documented competency assessment prior to performing the task unsupervised.
- ✓ The workers who implement behaviour support plans must maintain up-to-date Knowledge and understanding of regulated restrictive practices as described in the **National Disability Insurance Scheme (Restrictive Practices and Behaviour Support) Rules 2018** and **Disability Inclusion Regulation 2014, and The Senior Practitioner Act 2018.**

- ✓ Workers engages in ongoing professional development in the field of behaviour support, that is inclusive of evidenced-based support and best practice standards.
- ✓ The workers who implement behaviour support plans must maintain up-to-date knowledge and understanding of positive behaviour support, restricted practices and regulated restrictive practices.
- ✓ All workers who are responsible for the implementation of Behaviour Support plans must remain abreast of all developments, practices, and regulations associated with the use of restrictive practices.
- ✓ On-the-job training, internal or external courses, support for research and fieldworks, conference and seminar attendance, networking and mentoring programs relevant to behaviour support are available to workers as a part of training and development methods.
- ✓ All workers must undergo comprehensive training and education in the application of safe restriction, the associated laws, carer obligations, and the risks associated with restrictive practices before they are permitted to use such practices on a participant.
- ✓ Also, the training plan will include the identification of risks including actions and escalations.
- ✓ Records of induction, Mandatory Checks training and organisational and professional development provided to all Behaviour Support Practitioner and workers will be kept on each worker's record and on **Form09.Training Matrix** or worker's folder.

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- ✓ The Behaviour Support Practitioner and workers will be notified by the **Management Team** to complete their refresher training in these areas regularly and keep track of the workers training currency through **Form09. Training Matrix form.**
- ✓ Every worker and **Management Team** member would be able to have the opportunity of participation in Behaviour Support training and development activities.
- ✓ On-the-job training, internal or external courses, support for research and fieldworks, conference and seminar attendance, networking and mentoring programs relevant to behaviour support are available to workers as a part of training and development methods.
- ✓ Performance Reviews will motivate workers to play an active role in their ongoing improvement by identifying their training and development needs in consultation with their manager using Worker Performance Assessment Form.
- ✓ Training will relate specifically to the service users' needs, type of Behaviour Support and covers any specific support requirements the service user may require.
- ✓ All Behaviour Support Practitioner will have a working knowledge of relevant current legislation, national guidelines, organisational policies and procedures.

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3- Regulated Restrictive Practices Policy & Procedure

3.1. Purpose

✓ The purpose of this policy & procedure is that each participant is only subject to a regulated restrictive practice that meets any state and territory authorisation requirements and the relevant requirements and safeguards outlined in Commonwealth legislation and policy.

3.2. Scope

✓ This document applies to:

- All **One Life Foundation** staff and workers, whether permanent or casual, contractors, volunteers or business partners.
- All participants and their families

3.3. Definitions

✓ As per Applicable Practice Standards Policy & Procedure

3.4. Relevant Documents

- **NDIS Quality and Safeguarding Framework**
- **National Disability Insurance Scheme Act 2013.**
- **National Disability Insurance Scheme (Restrictive Practices and Behaviour Support) Rules 2018**
- **National Disability Insurance Scheme (Provider Registration and Practice Standards) Rules 2018**
- **National Disability Insurance Scheme (Incident Management and Reportable Incidents) Rules 2018.**

3.5. Policy & Procedure

3.5.1. Human rights policy

✓ People with disability have equal rights to the other members of society, including but not limited to the right to:

- Respect for inherent dignity, individual autonomy including the freedom to make one's own choices, and independence of persons
- Equality before the law and to equal protection under the law, without discrimination
- Liberty and security of the person
- Freedom from torture or cruel, inhuman or degrading treatment or punishment
- Freedom from exploitation, violence and abuse
- Respect for his or her physical and mental integrity on an equal basis with others
- Personal mobility with the greatest possible independence
- Freedom of expression and opinion and access to information
- The highest attainable standard of health without discrimination on the basis of disability
- Attain and maintain maximum independence, full physical, mental, social and vocational ability, and full inclusion and participation in all aspects of life
- An adequate standard of living for themselves and their families, and to social protection without discrimination on the basis of disability

3.5.2. Privacy, Dignity and Respect

✓ In all examinations, operations and interventions the following policies must be followed:

- Culture, diversity, Values and Beliefs Policy
- Privacy and Confidentiality Policy
- Decision -Making Policy
- Violence, Abuse, Neglect, Exploitation Policy
- Bullying, Harassment and Discrimination Policy

3.5.3. Regulated Restrictive Practices

✓ Under the NDIS (Restrictive Practices and Behaviour Support) Rules there are five categories of restrictive practice:

a) Seclusion, which is the sole confinement of a person with disability in a room or a

physical space at any hour of the day or night where voluntary exit is prevented, or not facilitated, or it is implied that voluntary exit is not permitted.

b) Chemical restraint, which is the use of medication or chemical substance for the primary purpose of influencing a person's behaviour. It does not include the use of medication prescribed by a medical practitioner for the treatment of, or to enable treatment of, a diagnosed mental disorder, a physical illness or a physical condition.

c) Mechanical restraint, which is the use of a device to prevent, restrict, or subdue a person's movement for the primary purpose of influencing a person's behaviour but does not include the use of devices for therapeutic or non-behavioural purposes.

d) Physical restraint, which is the use or action of physical force to prevent, restrict or subdue movement of a person's body, or part of their body, for the primary purpose of influencing their behaviour. Physical restraint does not include the use of a hands-on technique in a reflexive way to guide or redirect a person away from potential harm/injury, consistent with what could reasonably be considered the exercise of care towards a person.

e) Environmental restraint, which restrict a person's free access to all parts of their environment, including items and activities.

3.5.4. One Life Foundation and Staff Responsibilities

✓ **One Life Foundation** will:

- Ensure that all workers attend appropriate training and undergo documented competency assessment prior to performing the task unsupervised.
- Ensure that workers who are responsible for the implementation of Behaviour Support plans have Knowledge and understanding of regulated restrictive practices as described in the National Disability Insurance Scheme (Restrictive Practices and Behaviour Support) Rules 2018, and The Senior Practitioner Act 2018.
- Ensure that each participant and, with the participant's consent, their support network, providers implementing behaviour support plans, and other relevant stakeholders are engaged in discussions about the need for restrictive practices and they understand the risks associated with their use.

- Ensure that alternatives to the use of restrictive practices are promoted as part of these discussions.
- Ensure that professional services are provided to people with any challenging behaviour.
- Ensure that a positive interaction is promoted through support provision,
- Ensure that everyone is safe from injury.
- Ensure that behaviour intervention is being reviewed regularly and effectiveness of current approaches aimed at reducing and eliminating restrictive practices.
- For identifying the needs of participants, significant people from their family are involved.
- Ensure that Participant's development is promoted through implementation of all procedure.

✓ **One Life Foundation is committed** to meet conditions of registration for restrictive practice. The conditions include:

- A restrictive practice can only be used when it is part of a behaviour support plan developed by an behaviour support Practitioner whom the NDIS Quality and Safeguards Commissioner considers suitable.
- If a restrictive practice is used it must:
  - Be the least restrictive response possible in the circumstances
  - Reduce the risk of harm to the person or others
  - Be used for the shortest possible time to ensure the safety of the person or others

✓ **One Life Foundation** is dedicated to ensuring the following:

- Restrictive practices are conducted in a transparent and accountable manner.
- Restrictive practices are not employed as a means of punishing a participant or in response to behaviours that do not cause, or have the potential to cause, harm to the participant or others.
- The primary emphasis is placed on decreasing the extent, frequency, duration of any behaviours that may harm the participant or others; and
- A focus is placed on attempting to minimise or eradicate the need to implement any restrictive practices.

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3.5.5. Restrictive Practices process

- ✓ All providers and participants in Australia that have switched to the full NDIS scheme are now required to ensure that all their processes and actions in relation to restrictive practices are fully compliant with the provisions detailed in the NDIS Commission's regulatory framework and any additional legislative framework that is in place within their respective state.

- ✓ If participants express or display concerns about the use of restrictive practices, staff members must clearly communicate with them, their supporters, and other service providers, why the practices are required, and the risks associated with them. During these conversations, alternative approaches to restrictive practices must be fully explored.

- ✓ **Form42.Behaviour Support Risk Assessment Form** will be used to identify the associated risks.

- ✓ **One Life Foundation** must take responsibility for ensuring that staff, participants, family members, and relevant decision makers have a full understanding of the behaviour support function of the NDIS Commission.

- ✓ Staff need to ensure that any information about restrictive practices that is shared with participants is done so in a form of communication that is aligned with their individual needs. Where required, written information should be provided in the first language of the participant. Furthermore, all communications should use basic language that the participant can understand. Where necessary, staff members should provide participants with advocates or interpreters. Interpreters would be available as below:

    - The Translating and Interpreting Service (TIS National) is an interpreting service provided by the Department of Home Affairs. https://www.tisnational.gov.au/

- ✓ If the participant does not have a guardian or support worker and does not have the ability to advocate for themselves, he or she should be provided with an appropriate advocate using Advocate Nomination Form.

4- Supporting the Assessment and Development of Behaviour Support Plans Policy & Procedure

4.1. Purpose

✓ The purpose of this policy & procedure is that each participant's quality of life is maintained and improved by tailored, evidence-informed behaviour support plans that are responsive to their needs.

4.2. Scope

✓ This document applies to:

- All **One Life Foundation** staff and workers, whether permanent or casual, contractors, volunteers or business partners.
- All participants and their families

4.3. Definitions

✓ As per Applicable Practice Standards Policy & Procedure

4.4. Relevant Documents

- **NDIS Quality and Safeguarding Framework**
- **National Disability Insurance Scheme Act 2013.**
- **National Disability Insurance Scheme (Restrictive Practices and Behaviour Support) Rules 2018**
- **National Disability Insurance Scheme (Provider Registration and Practice Standards) Rules 2018**
- **National Disability Insurance Scheme (Incident Management and Reportable Incidents) Rules 2018.**
- **Form42.Behaviour Support Risk Assessment Form**
- **Interim behaviour support plan template**
- **Comprehensive behaviour support plan template**

4.5. Policy & Procedure

4.5.1. Specialist Behaviour Support Provider

✓ In situations in which a participant exhibits behaviours of concern that warrant a positive behaviour support approach without a requirement for the application of restrictive practices, a **Behaviour Support Plan** should be produced for that participant. If a participant exhibits behaviours that may pose a risk to his or her safety or the others, regulated restrictive practice will be required, and a **Behaviour Support Plan** must be developed using **behaviour support plan template.**

✓ **Form42.Behaviour Support Risk Assessment Form** will be used to identify the associated risks.

✓ **One Life Foundation** will work directly in collaboration with the specialist behaviour support provider who is responsible for creating the Behaviour Support Plan. The specialist behaviour support providers will ensure the following:

- Only behaviour support practitioners who have been assessed as suitable by the NDIS Commission will be used to provide services.
- All behaviour support plans are in full compliance with NDIS Commission requirements.
- All behaviour support plans are prepared in accordance with the findings of a detailed biopsychosocial assessment that incorporates a functional behavioural assessment.
- All behaviour support plans focus on decreasing and eradicating restrictive practices over a long-term basis.
- The responsibilities as to whom is responsible for implementing and reviewing the plan should be clearly delineated.

- All behaviour support plans are developed in consultation with the participant, his or her family or carers, and **One Life Foundation**.
- Any behaviours of concern are constructively reduced through the application of suitable evidence-based, behavioural strategies that include environmental considerations.
- All behaviour support plans are developed in a manner that is aligned with the requirements of the Commissioner; and
- All behaviour support plans are lodged with the NDIS Commission.

✓ In situations in which a detailed Behaviour Support Plan is required that includes restrictive practices, the behaviour support practitioner will be fully responsible for the following activities:

- Evaluating the participant's behaviours.
- Creating a Behaviour Support Plan.
- Reporting progress and outcomes via the ACT office of the senior practitioner and the NDIS online portal (NDIS commissioner); and
- Maintaining detailed records of the development and use of restrictive practices.

✓ All staff members who will be responsible for implementing plan must be considered within the plan, and their input solicited and incorporated.

✓ All relevant support staff should facilitate the acquisition of all data and information that are required to perform the functional behavioural assessment and any other evaluations that will be needed to produce the plan.

4.5.2. Proactive Behaviour Support strategies

✓ Proactive Behaviour Support strategies are as following:

- The primary goal of a person should be worked on to enhance the quality of life of a person as well as reduction of behaviour concern as a second goal
- The context that a behaviour of concern may occur as well as the environments that people are living in, should be considered
- Approaches to behaviour management that are inclusive and person-cantered
- Being committed to see the person's strength and potential
- Seeing the behaviour in the context of person's life as well as valuing them
- Being positive and believing that every behaviour has a purpose
- All behaviour support plans containing a regulated restrictive practice are provided to the Commissioner in the time and manner prescribed in the **National Disability Insurance Scheme (Restrictive Practices and Behaviour Support) Rules 2018.**

- Where regulated restrictive practices are to be included in a behaviour support plan, Regulated restrictive practice must:

(a) Be clearly identified in the behaviour support plan; and

- (b) State restrictive practice is to be used has an authorisation process in relation to that practice—be authorised in accordance with that process; and
- (c) Be used only as a last resort in response to risk of harm to the person with disability or others, and after the provider has explored and applied evidence-based, person-centred and proactive strategies; and
- (d) Be the least restrictive response possible in the circumstances to ensure the safety of the person or others; and
- (e) Reduce the risk of harm to the person with disability or others; and
- (f) Be in proportion to the potential negative consequence or risk of harm; and
- (g) Be used for the shortest possible time to ensure the safety of the person with disability or others.
- (h) Obtain agreement from any other providers implementing the plan, to follow it.

4.5.3. Consent

✓ The participant or his or her guardian must grant informed consent to implement the Behaviour Support Plan. The specific consent requirements that are in force in terms of the implementation of Behaviour Support Plans that contain the use of regulated restrictive practices are outlined in the and The Senior Practitioner Act 2018.

4.5.4. Registering Plans with the NDIS Commission

✓ Behaviour support plans that do not contain a regulated restrictive practice do not need to be lodged with the NDIS Commission.

✓ All Behaviour Support Plans contain regulated restrictive practices need to be registered with the NDIS Commission to ensure regulated restrictive practices are adequately monitored and full approval to put the plan in action needs to be secured. Once relevant behaviour support plan completed using **Interim behaviour support plan template** or **Comprehensive behaviour support plan template,** email the plan with other relevant documents to **behavioursupport@ndiscommission.gov.au** and

- ✓ Complete the behaviour support plan on a document. Use behaviour support plan template.
- ✓ Engage and support the implementing provider(s) as needed in obtaining authorisation.
- ✓ Include details of authorisation under the schedule of restrictive practices.
- ✓ Go to the NDIS Commission's portal https://www.ndiscommission.gov.au/
- ✓ You will need a PRODA account. Once you log in with your PRODA account request access for 'NQSC BSP Practitioner'. You will receive an email within approximately two business days confirming your approved access.
- ✓ Log in and select the 'Behaviour Support' tile. Click on 'Create plan' and select interim or comprehensive. Enter the person details, key contacts, details of start and end date of the plan, the providers who will be implementing the restrictive practices, behaviours of concern and the schedule of restrictive practices.
- ✓ Go to 'attachments' on the left-hand side navigation menu and attach this template.
- ✓ Go to the 'details' tab and click on 'send draft'.
- ✓ Further guidance material about how to lodge the plan is available on NDIS Commission webpage.

4.5.5. Authorising a regulated restrictive practice in ACT

- ✓ The Senior Practitioner holds an executive position in the ACT Government and has powers and functions provided by the Senior Practitioner Act 2018 ('the SP Act').

- ✓ The SP Act provides a legislative framework for the reduction and elimination of restrictive practices. The Senior Practitioner has independent oversight of restrictive practices used by providers of education, education and care, care and protection of children, and disability services.

- ✓ Section 7 of the SP Act defines a restrictive practice as a practice that is used to restrict the rights or freedom of movement of a person for the primary purpose of protecting the person or others from harm, and

  includes:

  - ➢ Chemical restraint.

- Environmental restraint.
- Mechanical restraint.
- Physical restraint.
- Seclusion.
- or > verbal directions, or gestural conduct, of a coercive nature.

✓ The role of the Senior Practitioner is to guide decision making and promote positive alternatives to restrictive practices that preserve a person's rights and freedoms. Under Section 10 of the SP Act, a provider must not use a restrictive practice on a person, other than in accordance with a registered Positive Behaviour Support (PBS) Plan. A restrictive practice can only be used outside of a registered PBS Plan when each of the following applies:

- the provider or relevant person for the provider believes on reasonable grounds that it is necessary to use the restrictive practice to avoid imminent harmto the person or others.
- the restrictive practice is the least restrictive of the person as is possible in the circumstances having regard to the kinds of restrictive practice that may be used, how it is applied, and how long it is applied for.
- if practicable – the use of the restrictive practice is authorised by the person in charge of the provider. The intent of the legislation is to ensure that restrictive practices are only used:
- as a last resort, for the shortest possible time and only when necessary to prevent harm to the person or others; and > if it is the least restrictive way of ensuring the safety of the person or others

4.5.6. PRINCIPLES OF POSITIVE BEHAVIOUR SUPPORT (PBS)

✓ The SP Act enshrines the principle that services must be provided in a way that involves positive behaviour support planning informed by evidence-based practice; and the implementation of strategies to produce behavioural change, focussed on skills development and environmental design (Section 9(2)(e)).

✓ Positive Behaviour Support (PBS) is an evidence-based framework to support people of all ages in all settings in reducing behaviours of concern (sometimes called challenging behaviours). It is the key strategy identified in the Act (Section 6) to maximise opportunities for achieving

positive outcomes and reducing or eliminating the need for restrictive practice. PBS is:

✓ Person-centred: ensuring the person's (or child's) life goals are at the centre of the process.

- a Partnership: collaborating with the person and all key stakeholders shapes the process of change.
- Planned: creating a clear document to ensure shared understandings and accountability.
- Positive: focusing on preventative, rather than reactive, strategies.
- Proactive: placing the responsibility for changing behaviour on both the person and their supporters.
- Purposeful: using a functional behavioural assessment approach to identify the reason for the behaviour.
- Process driven cycling iteratively through aprocess of identifying, assessing, planning, implementing, monitoring and evaluating data.

PBS has two main aims:

- to increase the person's quality of life, and
- to decrease the intensity, frequency and duration of behaviour that causes harm to the person or others

4.5.7. How to have a PBSP approved by the Central Panel

✓ Further information about each of these steps, as well as provider responsibilities in relation to the ongoing monitoring and review of an approved PBSP, is provided below.

✓ Contact actseniorpractitioner@act.gov.au and 6205 2811 to schedule a panel.

Step 1: Application for panel approval

✓ A provider must prepare a PBSP for a person and give the plan to the panel for approval. This must occur approximately one month prior to the Central Panel meeting (Step 3).

✓ The application will include:

- a completed PBSP Approval Panel template.
- a copy of the PBSP, and.
- all supporting document relevant to the restrictive practice (e.g., reports from therapists, doctors or psychologists, risk assessments, photographs of equipment).

Step 2: Notification that documents have been received by the panel

✓ Approximately two weeks prior to the panel meeting, the provider (applicant) will be notified that the application and supporting documentation have been received by the Central Panel. The Central Panel may also request additional information or documentation.

The applicant will also be notified of a time to meet with the Central Panel to discuss the PBSP.

Step 3: Central Panel meets to consider PBSPs

The Central Panel will meet on a monthly basis.

✓ The Central Panel will endeavour to have a two-way, mutually educative discussion with all parties about the PBSP. The tone is intended to be relaxed and collaborative. The Panel will give verbal feedback during the meeting.

✓ Consideration of each PBSP will take approximately one hour.

✓ It is important to note that the author/s of the **PBSP must be available to attend the meeting**, either face-to-face or via teleconference (by prior agreement). Other relevant parties, such as the person who is the subject of the PBSP, their family, carers and staff may also attend the Central Panel meeting.

✓ In discussion with the applicant, the Central Panel will assess the PBSP and decide whether or not to approve it. The panel must be satisfied any restrictive practice included in the PBSP:

- is necessary to prevent harm to the person or harm to others, and.
- will be used by the provider only in very limited circumstances, as a last resort and in the least restrictive way and for the shortest time possible in the circumstances.

✓ Where possible the Central Panel will reach decisions through consensus.

Step 4: Panel decision communicated to the provider

✓ The Central Panel will give the applicant written notice of its decision within one week of meeting to consider the application to approve a PBSP.

✓ The Central Panel's decision will be communicated on the Plan Approval Form, as either:

- *Outright Approval* - The proposed use of the restrictive practice within a PBSP is approved, for review at a future date not longer than 12 months.
- *Interim Approval* – The proposed use of the restrictive practice within a PBSP is approved. However, the panel has placed restrictions, requirements or clarifications on the approval which must be adhered to. A revised PBSP must be developed and submitted to the Central Panel by the stipulated date.
- *Not Approved (Resubmit)* - The proposed use of the restrictive practice is not approved. Guidance is provided and recommendations made as to the necessary changes that will need to be made before approval can be given.
- *Not Approved (Outright)* – The proposed use of restrictive practice is not approved.

Step 5: Approved PBSP forwarded to the Senior Practitioner for registration

✓ Once approved, the Central Panel will forward the PBSP to the Senior Practitioner for registration within 28 days, in accordance with the requirements of the Act.

✓ As part of this process, the Senior Practitioner may request further information from the Panel or applicant to be satisfied any restrictive practice included in the PBSP:

- is necessary to prevent harm to the person or harm to others, and.
- will be used by the provider only in very limited circumstances, as a last resort and in the least restrictive way and for the shortest time possible in the circumstances.

✓ The Senior Practitioner will send to the provider, the Public Advocate (if the person is under 18) and the plan author:

- a copy of the approved plan; and
- the plan registration number.

✓ It is the responsibility of the provider to ensure a copy of the PBSP is given to the person who is the subject of the plan (in an appropriate format) and the person's parent or guardian (if applicable).

Step 6: Ongoing regular monitoring and review of plans by providers

✓ Under the Act, providers are required to monitor and make a record of any restrictive practices, whether there is a positive behaviour support plan in place for the person or not.

✓ When should the reports be forwarded to the Senior Practitioner?

Restrictive practice	When to report
Routine restrictive practices (identified within a PBSP)	By the fifth day (5 days) after the end of the month.
PRN (as needed) restrictive practices (identified within a PBSP)	By the fifth day (5 days) after the end of the month.
Emergency restrictive practices (not identified within a PBSP)	Within 24 hours of the event.

In addition, the approved PBSP must be monitored and reviewed monthly by the provider to ascertain whether the restrictive practice is still required.

Step 7: Plan to be reviewed before expiry

✓ Under the Act, a registered PBSP expires 12 months after the day the plan is registered. However, as described above, the Central Panel may give approval for a shorter interim period.

✓ The purpose of a PBSP is to provide a formal framework for the reduction and elimination of restrictive practices over time. Prior to submitting a new PBSP to the Panel, providers are expected to use the data collected, review the plan and provide evidence that the use of a restrictive practice:

- is still necessary to prevent harm to the person or harm to others, and.
- will be used by the provider only in very limited circumstances, as a last resort and in the least restrictive way and for the shortest time possible in the circumstances.

5- Behaviour Support Plan Implementation Policy & Procedure

5.1. Purpose

✓ The purpose of this policy & procedure is that each participant's behaviour support plan is implemented effectively to meet the participant's behaviour support needs.

5.2. Scope

✓ This document applies to:

- All **One Life Foundation** staff and workers, whether permanent or casual, contractors, volunteers or business partners.
- All participants and their families

5.3. Definitions

✓ As per Applicable Practice Standards Policy & Procedure

5.4. Relevant Documents

- **NDIS Quality and Safeguarding Framework**
- **National Disability Insurance Scheme Act 2013.**
- **National Disability Insurance Scheme (Restrictive Practices and Behaviour Support) Rules 2018**
- **National Disability Insurance Scheme (Provider Registration and Practice Standards) Rules 2018**
- **National Disability Insurance Scheme (Incident Management and Reportable Incidents) Rules 2018.**

5.5. Policy & Procedure

5.5.1. Use of Restrictive Practices as per an Approved Plan

✓ Staff must not use restrictive practices unless **One Life Foundation** has established and documented in-depth process that outlines how the restriction will be performed in the setting of interest).

✓ Before a restrictive practice can be used, there is a requirement for it to be:

- Explicitly outlined in the participant's Behaviour Support Plan.
- Used only as a last resort in alignment with person-centred, evidence-based, and proactive strategies after all alternative options have been considered in depth.
- The least restrictive response feasible in the conditions to preserve the safety of the participant and others.
- Detailed in a Behaviour Support Plan that is lodged with the NDIS Commission.
- Used only in situations in which the participant's behaviour poses a risk to him- or herself, or others.
- Focused on decreasing the participant's risk of harming him- or herself or others.
- Proportional to the prospective negative outcomes or threat of harm; and
- Employed for the shortest duration possible to safeguard the safety of the participant or others.

5.5.2. Implementation of the Behaviour Support Plan

✓ It is the responsibility of the **Managing Director** to ensure participant consent obtained and make sure that support staff members who are involved in the implementation of the Behaviour Support Plan have ready access to all documents related to guardians, substitute decision makers, or appointed Public Guardians.

✓ Per the legal requirements, **One Life Foundation**'s behaviour support practitioners will collaborate with other behaviour support practitioners, psychologists, and other relevant professionals during the implementation of the Behaviour Support Plans.

✓ Staff members who are responsible for implementing a Behaviour Support Plan that has been developed by a **One Life Foundation** behaviour support practitioner or alternative Registered Specialist Behaviour Support Provider must:

- Proactively collaborate with the relevant behaviour support practitioner to put the steps outlined in the plan into action and ensure support delivery is aligned with positive behaviour support and evidence-based practice; and
- Where required, work with alternative providers who are responsible for supporting the participant to ensure the strategies that are outlined in the Behaviour Support Plan and implemented as per the underlying intention.

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- ✓ During the process of implementing Interim Behaviour Support Plans, **One Life Foundation** must conduct an in-depth assessment of the risks that the participant poses to him- or herself and others and put in place suitable actions by which those risks can be minimised and mitigated using **Form42.Behaviour Support Risk Assessment Form**.

- ✓ It is imperative that all staff who are responsible for using positive behaviour strategies or implementing restrictive practices have undergone full training in their use and Training Matrix updated accordingly.

- ✓ Performance Reviews will motivate workers to play an active role in their ongoing improvement. Performance review will be conducted to ensure that workers are implementing strategies in the participant's behaviour support plan appropriately. For this purpose, Worker Performance Assessment Form will be used.

- ✓ **One Life Foundation** needs to actively help specialist behaviour support providers to train all relevant personnel in the application, recording, and management of the behaviour support strategies that are included in Behaviour Support Plans. This includes delivering positive behaviour support to make sure participants' plans are implemented effectively.

- ✓ In circumstances in which it is not possible for **One Life Foundation** to engage effectively with the alternative service providers who are responsible for implementing the actions outlined in the Behaviour Support Plan or the support such groups provide to participants is not aligned with the specifications of the Behaviour Support Plan, **One Life Foundation** must notify the NDIS Commission immediately.

- ✓ **Form102. ABC Recording Chart** (Antecedent-Behaviour-Consequence) is in place and us used in the field of behaviour analysis and psychology to understand and address problem behaviours, including those related to restrictive practices.

- ✓ When using **Form102. ABC Recording Chart** it is used in the context of restrictive practices:

- ✓ **Identifying the Problem Behaviour**: First, the ABC chart is used to identify and define the problem behaviour that necessitates the use of restrictive practices. This behaviour might include self-injury, aggression, property destruction, or other dangerous actions.

- ✓ **Antecedent**: The "A" in ABC stands for antecedent, which refers to events or conditions that occur immediately before the problem behaviour. It's important to record what happens right

before the behaviour to determine potential triggers. For example, the antecedent might be a specific request, a change in routine, or sensory overload.

- ✓ **Behaviour**: The "B" in ABC represents the behaviour itself. This is where you document the details of the problem behaviour, including its frequency, duration, intensity, and any specific characteristics.

- ✓ **Consequence**: The "C" in ABC refers to the consequences or events that follow the problem behaviour. These consequences can be either positive (reinforcement) or negative (punishment). In the context of restrictive practices, you would record what happens immediately after the behaviour, such as staff intervention, removal from the situation, or any attempts to provide comfort or redirection.

- ✓ By systematically recording information in the ABC chart over time, you can analyse patterns and identify potential triggers and consequences that may be contributing to the problem behaviour. This information can then be used to develop behaviour intervention plans (BIPs) that aim to reduce the need for restrictive practices by addressing the underlying causes of the behaviour.

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6- Monitoring and Reporting the Use of Regulated Restrictive Practices Policy & Procedure

6.1. Purpose

✓ The purpose of this policy & procedure is that each participant is only subject to a restrictive practice that is reported to the NDIS Commission.

6.2. Scope

✓ This document applies to:

- All **One Life Foundation** staff and workers, whether permanent or casual, contractors, volunteers or business partners.
- All participants and their families

6.3. Definitions

✓ As per Applicable Practice Standards Policy & Procedure

6.4. Relevant Documents

- **NDIS Quality and Safeguarding Framework**
- **National Disability Insurance Scheme Act 2013.**
- **National Disability Insurance Scheme (Restrictive Practices and Behaviour Support) Rules 2018**
- **National Disability Insurance Scheme (Provider Registration and Practice Standards) Rules 2018**
- **National Disability Insurance Scheme (Incident Management and Reportable Incidents) Rules 2018.**

6.5. Policy & Procedure

6.5.1. Monitoring and Reporting the Use of Regulated Restrictive Practices Process

- ✓ To ensure **One Life Foundation** continues to improve and develop in terms of the planning and delivery of its services, there is a requirement to ensure staff evaluate and manage the implementation and effectiveness of the strategies detailed in the Behaviour Support Plans on an ongoing basis.

- ✓ All reviews must be managed by a trained behaviour support practitioner, and he or she should ensure that all staff members who will be responsible for the implementation of the Behaviour Support Plan are consulted and involved in the planning process. The main goal of the reviews should be to identify methods of decreasing or eradicating any restrictive practices in response to changes in the participant's behaviour and his or her overall progress.

- ✓ Comprehensive Behaviour Support Plans must be formally reviewed at least every twelve (12) months.

- ✓ Where necessary, and in response to participant feedback behaviour monitoring, or observations, reviews may be performed more frequently.

- ✓ Reviews will also be essential if the clients change service providers or relocate to another state or territory.

- ✓ **One Life Foundation** must give monthly online reports to the Commissioner regarding the use of regulated restrictive practices using a PRODA account. A report under this section must:

  (a) be in the form approved by the Commissioner; and

  (b) include any information, and be accompanied by any documents, required by the Commissioner.

- ✓ All monitoring activities should be completed in collaboration with the participants, associated carers, and members of their support networks, and should take into consideration any data acquired by other service providers.

- ✓ Collaboration may involve working with other representatives on an informal basis, for example, by reviewing case notes or eliciting participant feedback, or may occur on a formal level through supervision, team meetings, or other stakeholder interactions.

- ✓ In addition, there may be some instances in which a Behaviour Support Plan is produced before the providers who are responsible for implementing the plan have been allocated. In such

circumstances, the plan will undergo review after suitable providers have been identified.

- ✓ All changes to the Behaviour Support Plans should be in direct response to participant feedback, the observations and evaluations of their support networks, and formal evaluations of the participant's behaviour.
- ✓ Updated Behaviour Support Plans must be lodged with the NDIS Commission and should not be implemented until they have been authorised and communicated to all individuals who are responsible for implementing the actions outlined in the plan.
- ✓ The NDIS Commission may perform random audits to review Behaviour Support Plans or may assess plans in response to complaints or queries.

7- Behaviour Support Plan Review Policy & Procedure

7.1. Purpose

✓ The purpose of this policy & procedure is that each participant has a current behaviour support plan that reflects their needs, and works towards improving their quality of life, reducing behaviours of concern, and reducing and eliminating the use of restrictive practices.

7.2. Scope

✓ This document applies to:

- All **One Life Foundation** staff and workers, whether permanent or casual, contractors, volunteers or business partners.
- All participants and their families

7.3. Definitions

✓ As per Applicable Practice Standards Policy & Procedure

7.4. Relevant Documents

- **NDIS Quality and Safeguarding Framework**
- **National Disability Insurance Scheme Act 2013.**
- **National Disability Insurance Scheme (Restrictive Practices and Behaviour Support) Rules 2018**
- **National Disability Insurance Scheme (Provider Registration and Practice Standards) Rules 2018**
- **National Disability Insurance Scheme (Incident Management and Reportable Incidents) Rules 2018.**

7.5. Policy & Procedure

7.5.1. Behaviour Support Plan Monitoring and Review process

✓ To ensure **One Life Foundation** continues to improve and develop in terms of the planning and delivery of its services, there is a requirement to ensure staff evaluate and manage the

implementation and effectiveness of the strategies detailed in the Behaviour Support Plans on an ongoing basis.

✓ The NDIS Commission Portal will be updated as follows:

- upload behaviour assessments and the outputs of any other assessment reports that are deemed to be of relevance.
- upload Behaviour Support Plans to the system.
- monitor and update any current Behaviour Support Plans.
- upload the outcomes of any assessments, including functional behaviour assessments; and/or
- allocate any implementing service providers to the Behaviour Support Plans.

✓ All reviews must be managed by a trained behaviour support practitioner, and he or she should ensure that all staff members who will be responsible for the implementation of the Behaviour Support Plan are consulted and involved in the planning process. The main goal of the reviews should be to identify methods of decreasing or eradicating any restrictive practices in response to changes in the participant's behaviour and his or her overall progress.

✓ **One Life Foundation** must give monthly reports to the Commissioner regarding the use of regulated restrictive practices using a PRODA account.

✓ Where necessary, and in response to participant feedback behaviour monitoring, or observations, reviews may be performed more frequently.

✓ Collaboration may involve working with other representatives on an informal basis, for example, by reviewing case notes or eliciting participant feedback, or may occur on a formal level through supervision, team meetings, or other stakeholder interactions.

7.5.2. Record Keeping

✓ The following information and data must be recoded:

(a) a description of the use of the regulated restrictive practice, including:
(i) the impact on to the person with disability or another person.
(ii) any injury to the person with disability or another person.
(iii) whether the use of the restrictive practice was a reportable incident; and
(iv) why the regulated restrictive practice was used.

(b) a description of the behaviour of the person with disability that led to the use of the regulated restrictive practice
(c) the time, date and place at which the use of the regulated restrictive practice started and ended
(d) the names and contact details of the persons involved in the use of the regulated restrictive practice
(e) the names and contact details of any witnesses to the use of the regulated restrictive practice
(f) the actions taken in response to the use of the regulated restrictive practice
(g) what other less restrictive options were considered or used before using the regulated restrictive practice
(h) the actions taken leading up to the use of the regulated restrictive practice, including any strategies used to prevent the need for the use of the practice

- ✓ Information and data collected and recorded by behaviour support practitioners and support workers must comply with the National Disability Insurance Scheme (Restrictive Practices and Behaviour Support) Rules 2018.
- ✓ Records shall be kept for seven years from the day the record is made.

8- Reportable Incidents involving the Use of a Restrictive Practice Policy & Procedure

8.1. Purpose

✓ The purpose of this policy & procedure is that Each participant that is subject to an emergency or unauthorised use of a restrictive practice has the use of that practice reported and reviewed.

8.2. Scope

✓ This document applies to:

- All **One Life Foundation** staff and workers, whether permanent or casual, contractors, volunteers or business partners.
- All participants and their families

8.3. Definitions

✓ As per Applicable Practice Standards Policy & Procedure

8.4. Relevant Documents

- **NDIS Quality and Safeguarding Framework**
- **National Disability Insurance Scheme Act 2013.**
- **National Disability Insurance Scheme (Restrictive Practices and Behaviour Support) Rules 2018**
- **National Disability Insurance Scheme (Provider Registration and Practice Standards) Rules 2018**
- **National Disability Insurance Scheme (Incident Management and Reportable Incidents) Rules 2018.**
- **Form04.Incident Report**

8.5. Policy & Procedure

8.5.1. Reportable Incidents

✓ All unplanned or unapproved application of a restrictive practice represents a reportable incident that **One Life Foundation** must report to the NDIS Commission.

- ✓ Follow Incident Management Policy and Procedure and use **Form04.Incident Report**.
- ✓ Safety representative is responsible for reporting incidents that are reportable incidents to the Commissioner. In addition, any key personnel can notify Commissioner of reportable incidents in compliance with the National Disability Insurance Scheme (Incident Management and Reportable Incidents) Rules 2018 and Incident Management Policy and Procedure as per Incident Management Procedure.
- ✓ Following a reportable incident, it is the responsibility of the **Managing Director** to ensure that all participants and associated stakeholders are provided with adequate support through a Behaviour Support Plan that incorporates the use of restrictive practices.
- ✓ This includes the need to make sure that any participants who were involved in a reportable incident are promptly referred to a medical practitioner for evaluation where required.
- ✓ If the participant did not have a Behaviour Support Plan prior to the reportable incident taking place, the behaviour support practitioner may be required to put one in place.
- ✓ If a Behaviour Support Plan had been developed, it should be reviewed immediately to ensure that the provisions contained therein are adequate and effective. If the Behaviour Support Plan needs to revise, it must be done so in accordance with the registration and authorisation processes outlined above before any new actions or strategies are implemented. as
- ✓ In the aftermath of a reportable incident, **One Life Foundation** will work closely with all necessary mainstream services, including emergency services, medical practitioners, the police, and additional professionals, to ensure the response to the incident is timely, adequate, and effective.
- ✓ All reportable incidents must also be assessed in accordance with **Risk Management Policy and Procedure** and **Incident Management Policy and Procedure.**
- ✓ The findings of any reviews of this nature should be applied in a subsequent review of the Behaviour Support Plan of the participant concerned. Where necessary, additional support should be made available to the participant.
- ✓ The participant together with relevant stakeholders from their support must be directly involved in any evaluations that will directly impact their care and the review of reportable incidents that have had an impact on them.

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- ✓ In a situation in which an unauthorised restrictive practice has been employed, the members of staff who were involved, in addition to **Managing Director** and other relevant providers, must collaborate with one another to develop an action plan to prevent the incident from recurring and ensure all managements strategies that are implemented in the future are aligned with the participant's needs.
- ✓ All meetings in which unauthorised restrictive practice is discussed should be fully minuted using Meeting Minutes Template and the outputs shared with all relevant parties.

9- Interim Behaviour Support Plans Policy & Procedure

9.1. Purpose

✓ The purpose of this policy & procedure is that Each participant with an immediate need for a behaviour support plan receives an interim behaviour support plan based on evidence-informed practice, which minimises risk to the participant and others.

9.2. Scope

✓ This document applies to:

- All **One Life Foundation** staff and workers, whether permanent or casual, contractors, volunteers or business partners.
- All participants and their families

9.3. Definitions

✓ As per Applicable Practice Standards Policy & Procedure

9.4. Relevant Documents

- **NDIS Quality and Safeguarding Framework**
- **National Disability Insurance Scheme Act 2013.**
- **National Disability Insurance Scheme (Restrictive Practices and Behaviour Support) Rules 2018**
- **National Disability Insurance Scheme (Provider Registration and Practice Standards) Rules 2018**
- **National Disability Insurance Scheme (Incident Management and Reportable Incidents) Rules 2018.**
- **Interim Behaviour Support plan**

9.5. Policy & Procedure

✓ Where restrictive practices may need to be rapidly implemented in response to situations

where there is a clear and immediate risk of harm linked to behaviour(s) of concern and there is no behaviour support plan in place, an **Interim Behaviour Support plan** must be developed within one month.

- ✓ The purpose of the interim behaviour support plan is to mitigate the risk of harm to the people around participants.
- ✓ The Interim Behaviour Support Plan should be developed by a registered behaviour support practitioner and should include details of behaviour support any required regulated restrictive practices that include the following:
    - strategies by which the onset of the behaviour that is of concern can be managed.
    - the interventions that will be employed if the behaviour of concern starts to escalate.
    - the strategies by which the behaviour of concern can be managed as a means of deescalating it and bringing the incident to an end as quickly and safely as possible; and
    - how all information relevant to the restrictive practice will be recorded.
- ✓ As is the case with restrictive practices in general, any restrictive practices that are employed should be limited to the lowest degree of force or restriction required and should only be enforced for the time necessary to mitigate an apparent risk.
- ✓ **One Life Foundation** will collaborate with relevant mainstream services such as the Police, other emergency services, mental health and emergency departments, treating medical practitioners and other allied health clinicians to contribute to the plan's development.
- ✓ **One Life Foundation** will take all reasonable steps to facilitate the development of an interim behaviour support plan for participant that covers the use of the practice within 1 month after the first use of the regulated restrictive practice.
- ✓ Adequate training will be provided to the workers in the implementation of the interim behaviour support plan and training matrix will be updated accordingly.
- ✓ For every participant, the following actions should be taken within one (1) month:
    - Consent should be obtained.

- The **Managing Director** needs to review the content of the interim plan, verify all behaviour support strategies outlined therein, and be satisfied that the strategies contained within the plan signify the least restrictive of the choices that are available that have a sufficient evidence base concerning the management of the risk.
- The **Managing Director** should stipulate how long the interim plan is authorised for. This should be the least duration required to ensure the risk is sufficiently managed and must not extend beyond five (5) months; and
- The **Managing Director** must report any uses of restrictive practices to the NDIS Quality and Safeguards Commission fortnightly throughout the time for which the interim plan is authorised.

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